

Sample Hospital

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DISCHARGE SUMMARY

Patient: Doe, Anna B.	DOB: 03/15/1958	MRN: SH-2024-088451
Admission Date: 01/08/2025	Discharge Date: 01/12/2025	Attending: Dr. Jane Smith, MD
Service: Internal Medicine	Room: 4-West, Bed 412B	Insurance: Sample Insurance Company

CHIEF COMPLAINT

Shortness of breath and bilateral lower extremity edema worsening over 5 days.

HISTORY OF PRESENT ILLNESS

Mrs. Doe is a 66-year-old female with a past medical history significant for congestive heart failure (EF 35%), type 2 diabetes mellitus, hypertension, and chronic kidney disease stage III who presents with progressive dyspnea on exertion and worsening bilateral pedal edema over the past 5 days. Patient reports weight gain of approximately 8 lbs over this period. She admits to dietary indiscretion over the holidays, consuming high-sodium foods. She denies chest pain, palpitations, syncope, or fever. Her home medications include furosemide 40mg daily, lisinopril 20mg daily, carvedilol 12.5mg BID, and metformin 1000mg BID. She reports compliance with all medications except missed 2 doses of furosemide last week due to running out of refills.

HOSPITAL COURSE

Patient was admitted to the cardiology service and started on IV furosemide 80mg BID with strict I&Os; and daily weights. Telemetry monitoring showed no arrhythmias. Echocardiogram on 01/09 confirmed EF 33% (stable from prior), moderate mitral regurgitation, and mild tricuspid regurgitation. BNP on admission was 1,847 pg/mL, trending down to 642 pg/mL by day 3. Creatinine remained stable at 1.4-1.6 mg/dL throughout admission. Patient was diuresed 4.2 liters total over 4 days with significant improvement in symptoms. Transitioned to oral furosemide 60mg BID on day 3 with continued response. By discharge, patient was ambulating without dyspnea, edema reduced to trace bilateral, and oxygen saturation 96% on room air.

DISCHARGE DIAGNOSES

ICD-10	Description	Type
I50.22	Chronic systolic heart failure, acute decompensation	Primary
E11.65	Type 2 diabetes mellitus with hyperglycemia	Secondary
I10	Essential hypertension	Secondary
N18.3	Chronic kidney disease, stage III	Secondary
I34.0	Mitral valve insufficiency	Secondary

MEDICATIONS AT DISCHARGE

Medication	Dose	Route	Frequency	Instructions
Furosemide	60 mg	PO	BID	Take with food; weigh daily

Lisinopril	20 mg	PO	Daily	Hold if SBP < 90
Carvedilol	12.5 mg	PO	BID	Take with meals
Metformin	1000 mg	PO	BID	Take with meals
Potassium Chloride	20 mEq	PO	Daily	While on furosemide
Aspirin	81 mg	PO	Daily	Enteric coated
Atorvastatin	40 mg	PO	QHS	Take at bedtime

FOLLOW-UP INSTRUCTIONS

1. Cardiology follow-up with Dr. Smith in 7 days (appointment scheduled 01/19/2025)
2. Daily weights — call clinic if weight increases >3 lbs in 24 hours or >5 lbs in one week
3. Sodium-restricted diet (<2000 mg/day)
4. Fluid restriction: 1.5 liters daily
5. Lab draw (BMP, BNP) on 01/15/2025 at outpatient lab
6. Return to ED if: chest pain, severe shortness of breath, dizziness/fainting, or weight gain as above

CONDITION AT DISCHARGE

Stable. Improved. Ambulating independently.

Jane Smith, MD, FACC

Department of Cardiology

Electronically signed 01/12/2025 14:32 CST

SYNTHETIC DATA — FOR DEMONSTRATION PURPOSES ONLY — NO REAL PATIENT INFORMATION